

Foundayo

Brand-Name GLP-1 · Provider Reference

Weight Loss Program · KORB Health Group · korb-glp1-data.js v1.2 · generated 2026-08-09

Foundayo is written as a Tebra STANDARD prescription, never Tebra Compound. KORB does not manage fulfillment, does not complete prior authorizations, and does not handle coupons.

Available states

TX, CA, AL, WI

Brand-name GLP-1 through the manufacturer programs is only available in Texas, California, Alabama and Wisconsin. Source: the brand prescribing document. This is a program limit, not a shipping limit — do not offer a brand pathway to a patient outside these four states.

Rules and visit fee

Order via	Tebra Standard prescription (NOT Tebra Compound)
Billing	Patient pays the manufacturer program or the local pharmacy directly.
Prior authorizations	KORB does NOT complete insurance prior authorizations.
Coupons and savings cards	KORB does NOT handle coupons or savings cards.
Patient responsibility	KORB does not complete insurance prior authorizations and does not handle coupons or manufacturer savings cards. The patient is responsible for checking pricing, coverage and every other aspect of the medication.
Local pharmacy	A brand prescription can be sent to the patient's local pharmacy on request, under the same rules. KORB will not re-send a prescription between pharmacies to chase a lower price. The patient does that comparison before ordering.
Prescription visit fee	\$79 — charge code FITGLP1001
Fee covers	Cash-pay only. The fee covers the clinical visit, prescription issuance and program administration. Medication cost, pharmacy processing, shipping and supplies are set by the external pharmacy and paid directly by the patient.
Non-refundable	The visit fee is non-refundable once the visit has occurred or a prescription has been issued. Medication cost is not included.

Medication pricing is deliberately not listed

Manufacturer pricing changes without notice and KORB does not control it. Storing it creates a maintenance burden and risks providers quoting incorrect pricing to patients. Price and coverage are the patient's responsibility.

Foundayo — Oral tablet

Route: oral • **Frequency:** once daily

Order via: Tebra Standard prescription

Pharmacy: LillyDirect Self Pay Pharmacy Solutions, 4343 Equity Dr, Columbus, OH 43228-3842 • (833) 432-4322

Titration: Start 0.8 mg once daily. Increase to 2.5 mg after at least 30 days, then 5.5 mg after at least 30 days. Further increases to 9, 14.5 or 17.2 mg at minimum 30-day intervals based on response and tolerability. Maximum 17.2 mg daily.

Dispensing option	Quantity	Refill	Days	When to use
30-day supply	30	0	30	New prescription or any dose change. Use until the dose is stable.
60-day supply	60	0	60	Clinically stable patients only. Quantity 60, no refill.

Tebra STANDARD fields — 30-day supply

Dose	Drug (dropdown)	Patient Instructions	Qty	RF	Days
0.8 mg	Foundayo 0.8 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	30	0	30
2.5 mg	Foundayo 2.5 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	30	0	30
5.5 mg	Foundayo 5.5 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	30	0	30
9 mg	Foundayo 9 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	30	0	30
14.5 mg	Foundayo 14.5 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	30	0	30
17.2 mg	Foundayo 17.2 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	30	0	30

Tebra STANDARD fields — 60-day supply

Dose	Drug (dropdown)	Patient Instructions	Qty	RF	Days
0.8 mg	Foundayo 0.8 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	60	0	60
2.5 mg	Foundayo 2.5 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	60	0	60
5.5 mg	Foundayo 5.5 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	60	0	60
9 mg	Foundayo 9 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	60	0	60
14.5 mg	Foundayo 14.5 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	60	0	60
17.2 mg	Foundayo 17.2 mg tablet	Take 1 tablet by mouth once daily with or without food. Swallow whole. Do not break, crush, or chew.	60	0	60

- Once-daily oral GLP-1. No injection, no pen needles.
- Dose selection and titration follow FDA-approved labeling.

Is this the right patient

Program: **Metabolic Health & Weight Loss Program** · Visit cadence: **Every 4 to 8 weeks**

Indications

- Weight loss
- Medically driven weight loss
- Increased energy
- Better sleep
- Improved mental health
- Improved cardiovascular health

Identifying the appropriate candidate

Body mass index	BMI greater than 25 is overweight · BMI greater than 30 is obese
FDA-approved uses	• Weight loss • Type 2 diabetes • Prevention of cardiovascular death, myocardial infarction and stroke in adults with established cardiovascular disease who are overweight or obese
Diabetic and pre-diabetic patients	KORB treats weight loss. Patients who are diabetic or pre-diabetic are acceptable. The patient must continue diabetes management with their PCP — KORB does not manage diabetes and does not order or monitor labs for it.

Agent monograph — Orforglipron — oral non-peptide GLP-1 receptor agonist

Verify against current prescribing information

NEWEST AGENT IN THE PROGRAM. This monograph is the least certain in this document. Verify against the current FDA prescribing information before relying on any specific claim, particularly dosing intervals, contraindications and interactions.

Definition	A small-molecule, non-peptide GLP-1 receptor agonist taken orally once daily. Because it is not a peptide, it does not require the absorption enhancer that oral semaglutide depends on, and it is taken without the empty-stomach and water-volume restrictions that apply to oral semaglutide.
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Mechanism

- Binds and activates the GLP-1 receptor as a small molecule rather than a peptide analogue.
- Glucose-dependent insulin secretion and glucagon suppression.
- Slowed gastric emptying and central appetite reduction, as with the class.
- Not a peptide, so it needs no absorption enhancer and carries none of the empty-stomach or water-volume restrictions that apply to oral semaglutide.

Clinical evidence

- Supported by the phase 3 programme in obesity and type 2 diabetes.
- Evidence base is smaller and shorter than for semaglutide or tirzepatide, simply because the agent is newer.
- Long-term outcome data are correspondingly limited.

Compounded preparation

Brand product dispensed through the manufacturer programme. Not compounded. Fulfilment, payment and shipping are handled by the manufacturer, not KORB.

Contraindications

Absolute — do not initiate	Cautions — evaluate before initiating
• Personal or family history of medullary thyroid carcinoma (MTC)	• History of pancreatitis
• Multiple endocrine neoplasia syndrome type 2 (MEN2)	• Gastroparesis or other significant gastrointestinal motility disorder
• Known hypersensitivity to orforglipron or any component of the formulation	• Active gallbladder disease
• Pregnancy, breastfeeding, or planning pregnancy	• Severe renal impairment
	• History of suicidal ideation or active depression — monitor and ask

Medication interactions

• Delayed gastric emptying can alter absorption of concomitant oral medication.
• Insulin and sulfonylureas: additive hypoglycaemia risk.
• ANAESTHESIA AND PROCEDURES: as with the injectable GLP-1 agents, disclose use before any procedure requiring sedation.
• Other GLP-1 receptor agonists: do not combine.
• Verify the current prescribing information — the interaction profile for this agent is less established than for the older molecules.

Side effects

Common	Less common
• Pain, redness or swelling at the injection site	• Pancreatitis
• Nausea and vomiting	• Gastroparesis
• Diarrhea	• Bowel obstruction
• Stomach pain	• Gallstone attacks
• Constipation	
• Low appetite	
• Headaches	
• Dizziness	

Monitoring

■ Weight and BMI at each visit	■ Abdominal pain that is severe or radiates to the back — assess for pancreatitis
■ Ask about blood pressure — monitored by the PCP, not measured by KORB	■ Mood and mental health
■ Ask about recent HbA1c or glucose if diabetic — ordered by the PCP, not by KORB	■ Lean mass preservation — protein intake and resistance training
■ Gastrointestinal tolerance through titration	■ Adherence — daily dosing is easier to miss than weekly

Patient counseling

“This is a once-daily tablet that works on the same gut hormone pathway as the injectable medications, but it is a different kind of molecule, so you can take it with or without food and without the water restrictions that apply to some other oral options. Swallow it whole. Nausea is the most common side effect and is usually worst after a dose increase. It is newer than the injectables, so there is less long-term data. Tell any surgeon or anaesthetist that you take it.”

• Gastrointestinal side effects are most common during dose escalation.
• Slower titration or holding at the current dose may improve tolerability.
• Instruct the patient to report persistent, severe or worsening symptoms.

- New or severe symptoms may require dose adjustment, temporary hold, or discontinuation.
- Vials are good for 28 days from first use regardless of the pharmacy BUD. Counsel the patient to discard at 28 days even if medication remains.

Chart attestation

Patient counseled on orforglipron as an oral GLP-1 receptor agonist for chronic weight management. Mechanism, once-daily administration, expected time course, gastrointestinal side effects during titration, pancreatitis warning signs, the more limited long-term evidence base relative to the injectable agents, the need to disclose use before any procedure requiring sedation, and the monitoring and follow-up plan were reviewed. Patient verbalized understanding and consented to proceed.

ICD-10

DOCUMENTATION ONLY — NOT BILLING. Select the code matching the documented presentation.

Primary	Secondary
E66.9 — Obesity, unspecified	Z68.— — Body mass index (add the matching BMI code)
E66.01 — Morbid (severe) obesity due to excess calories	Z71.3 — Dietary counseling and surveillance
E66.3 — Overweight	Z72.3 — Lack of physical exercise

Escalation and discontinuation

Hard stop — discontinue immediately	Flag and reassess
• Suspected pancreatitis — severe persistent abdominal pain, often radiating to the back, with or without vomiting. Stop the medication and evaluate.	• Gastrointestinal side effects that do not settle within two weeks of a dose increase — hold at the current dose or step back rather than pushing forward.
• Hypersensitivity or anaphylaxis-type reaction.	• Right-upper-quadrant pain, or gallstone symptoms.
• Pregnancy identified or confirmed.	• Rapid or excessive weight loss, or loss of lean mass.
• Medullary thyroid carcinoma or MEN2 identified at any point.	• New or worsening depression, or any mention of suicidal thoughts.
• Persistent severe vomiting with dehydration or acute kidney injury.	• Worsening retinopathy in a diabetic patient during rapid glycaemic improvement.
	• Plateau with no further response at the maximum tolerated dose — reassess the plan rather than continuing indefinitely.
	• Patient scheduled for surgery or a procedure requiring sedation — coordinate holding.

• GLP-1 receptor agonists are not currently on the WADA prohibited list, so the athlete hard stop used in the Functional Health peptide program does not apply to this program. Confirm current WADA status before treating a tested competitive athlete.

Questions and escalation

- Pharmacy or shipping issues — Operations
- Charge codes and billing — Operations
- Clinical protocol questions — Clinical Director
- Corrections to this document — Clinical Operations

Do not improvise

- Do not substitute a pharmacy the tool marks as unavailable
- Do not alter quantity, refill or days supply
- Do not paraphrase patient instructions
- Do not quote pricing without confirming with Operations